

Bronchiolitis

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Updated by: L. Haut, MD, C. Showalter, MD, & L. Buenger, PharmD

Reviewed by: PEMCET

Presentation and diagnosis

- A common lung infection in infants and young children leading to congestion and inflammation of the bronchioles (small airways)
- Typically caused by a viral infection (RSV, COVID, rhino/enteroviruses, or many others)
- **Less than two years old** presenting with
 - **URI symptoms:** nasal congestion/rhinorrhea +/- fever, **AND**
 - **Lung findings:** wheezing and/or crackles on auscultation
- Peak symptoms typically day 4 or 5
- A clinical diagnosis that **does not routinely require viral panel for confirmation***

General treatment

- Nasal suctioning (bulb or suctioning with saline)
- Antipyretics/pain control: acetaminophen or ibuprofen (if 6 months or greater)
- Hydration:
 - preferably oral or NG
 - IV fluids if dehydrated/not taking great oral intake/not making good wet diapers

Supplemental treatment

- Oxygen via nasal cannula, up to 2L - mild increased work of breathing
 - Hypoxia
 - Persistent increased work of breathing
 - **Patient in gown to assess and make sure to reassess**
- High flow nasal canula
 - if moderate to severe distress (grunting, head bobbing, retractions)
 - Moderate to severe tachypnea
 - RR >70 for 0 to less than 1 year old
 - >50 for 1 to 2 years old

Treatment and testing that is not routinely needed

- **Routine chest X-rays are NOT NEEDED.** Consider if:
 - Atypical course
 - New fever following typical course, or prolonged fever
 - FiO₂ > 50%
- **Routine viral testing is NOT NEEDED.** Consider if:
 - RSV if less than 6 weeks of age
 - COVID if high risk
 - Flu if qualifies for oseltamivir (Tamiflu)
- **Routine antibiotics are NOT NEEDED.** Consider if secondary bacterial infection (Pneumonia, acute otitis media, etc.)
- **Routine use of nebulized hypertonic saline is NOT recommended.**
- **When to consider albuterol?**
 - Less than 36-week gestational age/bronchopulmonary dysplasia/chronic lung disease (CLD)
 - Strong family history of asthma/reactive airway disease
 - Patient with prior history of response to albuterol or regular use at home for CLD
- **Routine use of steroids (dexamethasone or prednisONE) is NOT needed.** Consider if strong and persistent response to albuterol
- **Routine use of antihistamines NOT RECOMMENDED.**
- **Honey for antitussive NOT RECOMMENDED less than 12 months.**

Disposition/When to admit

- Persistent moderate or severe increased work of breathing
- Supplemental O₂ requirement
- Poor hydration status
- High risk past medical history or social history
- Young age (less than 2 – 4 weeks adjusted)

References

Ralston SL, Lieberthal AS, Meissner HC, et al. Clinical practice guidelines: the diagnosis, management, and prevention of bronchiolitis. Pediatrics. 2014;134(5):e1474-e1502.



Riley Children's Health
Indiana University Health

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For referrals or consults, call **1.844.MyRiley**.

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